



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : حمديه سلمى السيد محمد
المحافظة : سوهاج الحى / المدينة : طهطا
رقم البطاقة : 27410162600384 رقم التليفون : 01229355852
العنوان : -
العمر (تاريخ الميلاد) : 16-10-1974 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 04-10-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج على نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني الوظيفة: -----
الاسم: د/ندى حسن الوظيفة: -----
الاسم: د/نرمين رمضان الوظيفة: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: حمديه سلمى السيد محمد
National ID: 27410162600384



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ ومساءة الخروج :
PM 01:17 05-07-2026

تاريخ ومساءة الدخول :
--:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Further investigations and imaging

الوقت :

--:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

حمديه
سلمى السيد محمد
قرية بلجا
مركز طهطا - سوهاج

١٩٧٤/١/١٦ ٢٧٤١٠١٦٢٦٠٠٣٨٤

JL4681434

٢٠٢٢/٠٧ ٢٧٤١٠١٦٢٦٠٠٣٨٤
ربة منزل

أنثى
مسلمة
متزوجة

عرفه عبدالرؤف عبدالعظيم مهدي
البطاقة سارية حتى ٢٠٢٩/٠٧/١١

١

Name	حنية سليم السيد	MRN	1575/2026
Age	50 y.o	ID	
Menopausal status	Post. Menop.	Phone	01145 219 371
NRS SCORE		Marital status	
BMI		Family history	the "Mother"
CO morbidity		REASON OF MDT	

Clinical Data

BX:

R

L

ER:

-ve

PR:

-ve

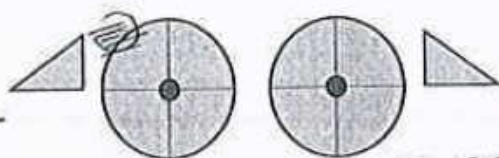
HER 2:

-ve

Ki67:

Sonomammography:

2020 → R breast
ET 4 M2 →



NACT → R+ BCS
AC-1st
TNM → PORT

Mets Workup:

10/2021 → Recurrence → Gem/Carbo →
Discontinue NACT
Started

Recommendations:

→ Maintenance Xeloda

9/2024 → Subpectoral recurrence
→ biopsy: TNBC GII

Taxol/carbo → SD

MDT OUTCOME & PLAN

11/2025 Shifted to Xeloda due to DVT
Newelox & difficult IV

1/2026 → clinical progression
for PORT port cath & IV chemo
CT CAP
Carbo/gem

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
			+ Consider Palliative RT to this Site after CTs		

Name: حمدي سلمى السيد		
MR. : 1377		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشر بأن نوضح فيما يلي تقريراً عن حالتها		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد أنها		
The patient is a 51-year-old individual who is pre-menopausal. The clinical history is notable for locally recurrent invasive ductal carcinoma (IDC) of the breast, triple-negative phenotype, with no BRCA1/2 mutation identified. The patient has previously undergone right-sided breast cancer treatment, including surgery, radiation, and multiple systemic regimens (gemcitabine/carboplatin, AC/Taxanes, taxol/carboplatin, capecitabine/oxaliplatin, and further gemcitabine/carboplatin). The most recent course involved right axillary radiation (30 Gy in 10 fractions completed on 11/5/2026) following progressive enlargement of a right axillary mass noted on imaging. Pathology confirms triple-negative breast cancer with wild-type BRCA status and a PD-L1 expression of 7 %. Imaging demonstrates a right upper chest wall mass measuring approximately 10 × 10 cm. MRI of the right shoulder performed on 21/7 reveals two large multilocular cystic lesions overlying the right subscapular region (approximately 93 × 80 mm) and an additional subcutaneous mass at the right root of the neck and shoulder measuring 10.6 × 5.4 mm. The disease is considered advanced axillary involvement, and the patient is not a candidate for further surgical intervention. The referring team recommended surgical referral, and the multidisciplinary team (MDT) requested MRI for further assessment. The MDT outcome and plan are to treat the loco-regionally recurrent triple-negative breast cancer with a new course of AC chemotherapy (adriamycin 40 mg/m² and endoxan 600 mg/m²) for 3–4 cycles, guided by response, with a maximum of four cycles to avoid exceeding cumulative dose limits.		
تحريراً في ,		
	التاريخ :	23-09-2026
	الوقت :	
	الطبيب المعالج :	
يعتمد ,		

Patient Name : الأستاذة / حندية سامي السيد محمد

Int No. : 27410162600384

No. : 3623/24

: Female

: 49 Y

Ref Doc :

Request No. : 13003481

Request Date : 25/09/2024

Verification Date : 08/10/2024

Financial Category : نفقة الدولة - أورام دار السلام

Department : Out Patient

Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Subpectoral lesion, known case of right breast cancer.
Description of Specimen: Tru-cut biopsy, collected at 24/9/2024.

Gross Examination:

Three grayish white tissue cores up to 1.5 cm, totally submitted.

Microscopic Examination:

Histological type: Recurrent Invasive duct carcinoma

Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+3+2) Grade III.

Ductal carcinoma in situ: No.

Perineural invasion: No.

Angioinvasion: No.

Microcalcifications: Not detected.

Additional pathologic findings: Lymphocytic infiltration 5%. Focal clearing and focal squamoid differentiation.

Diagnosis:

Subpectoral lesion, Tru-cut biopsy:

Recurrent Invasive duct carcinoma G III.

For further IHC (ER, PR, Her2, Ki67).

Signature

Dr. Salma Tarek.

Dr. Menna Nabil.

Verified by :

DR.Menna Nabil

Printing Date : 13/10/2024 10:07:10

Page 1 of 1

Printing By : marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mistr El Qadema - Cairo, Egypt

nt Name : الأستاذة / حمنة سالمى السيد محمد	Request No. : 13003481
nt No. : 27410162600384	Request Date : 25/09/2024
No. : 3523/24	Verification Date : 19/10/2024
: Female	Financial Category : أورام دار السلام - نفقة الدولة
: 49 Y	Department : Out Patient
rrreing Doc. :	IHC No. :
	Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

cal Data: Subpectoral lesion, known case of right breast cancer.
re of Specimen: Tru-cut biopsy, collected at 24/9/2024.

Unique:
ctions were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana)
stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and
atoxylin as counter stain. The sections treated against:
clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

ults:
F (Positive control showed appropriate reactivity):
R: No nuclear reaction in tumor cells (score 0/8).
R: No nuclear reaction in tumor cells (score 0/8).
er2: No membrane reaction in tumor cells (score 0).
67: 25% nuclear reaction of tumor cells.

Conclusion:
ppectoral lesion, Tru-cut biopsy:
current Invasive duct carcinoma, Grade III.

R: Negative (score 0/8).
R: Negative (score 0/8).
er2: Negative (score 0).
i67: Positive in 25% of tumor cells.

Scoring system: "ER, PR"

SCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1,

Verified by :
Dr. Sagda El-Kady

Printing Date : 18/10/2024 10:55:51

Page 1 of 2

Printed By : marwa mohamed yousse



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mistr El Qadema - Cairo, Egypt

Patient Name الأستاذ / حذيه بلقي السيد محمد
 Patient No. 27410162600384
 Path No. 3623/24
 Sex Female
 Age 49 Y
 Referring Doc.

Request No. 13003481
 Request Date 25/09/2024
 Verification Date 19/10/2024
 Financial Category أرقام دار السلام - نفقة الدولة
 Department Out Patient

Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
 Dr. Salma Tarek.
 Dr. Sagda Elkady.

Verified by :
 Dr. Sagda El-Kady

Printing Date : 28/10/2024 10:55:51

Page 2 of 2

Printed By : .marwa mohamed yousse



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
 Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Mixr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / د. عبد الله بن محمد
 Patient No. : 2741016260034
 Path No. : 1547/2022
 Sex : Female
 Age : 47 Y
 Referring Doc :

Request No. : 13008435
 Request Date : 06/06/2022
 Verification Date : 02/07/2022
 Financial Category : أورام دار السلام - فئة الأولى
 Department : Out Patient
 Registered by : Ali mostafa

Pathology Report

Amended Report Clinical Data:

Right WLE and ALND in 2019 revealed IDC Grade II then MACT then Radiotherapy ,
 Recurrence then NACT

Nature of Specimen:

Right salvage mastectomy + partial pectoralis major and minor excision + infraclavicular LN
 excision.
 Sample collected at 9/6/2022 ; 3:00pm

Gross Examination:

- Procedure: Salvage mastectomy specimen measuring 20x10x6cm with covering skin ellipse 20x10 cm & attached muscle 11x6 cm .
- Localization: Right breast
- Tumor extent
- o Number: an ill defined grayish white tumor bed site
- o Size: 4.5x4.5x2 cm
- o Skin involvement: Free gross
- Distance from margins: 0.7 cm deep margin
- Lymph nodes number and size: 6 LNs up to 1 x 0.5 cm

Microscopic examination:

- Tumor bed with totally burnt out tumor.
- Skin involvement: Free nipple and areola
- Lymph node involvement: 1/6 with no extracapsular extension.
- Resection Margins: Free deep and muscular margins
- Histologic response to therapy: Pathologic Partial Response (PPR) (TN1 100%)

Verified by :
 dr.Enas

Printing Date : 20/07/2022 10:26:42

Page 1 of 2

Printing By : DR.Israa Elger

Request No. : 13008435
Request Date : 06/06/2022
Verification Date : 02/07/2022
Financial Category : أورام دار السلام - فاقه الدولة
Department : Out Patient
Registered by : Ali mostafa

Diagnosis:

Right Breast cancer post NACT ; salvage mastectomy :
Tumor bed with totally burnt out tumor (TN1 100%) .
Free nipple and areola , free deep margin , free muscular margin.
Positive metastatic deposits in one lymph node (1/6) with no extracapsular extension.
Pathologic Partial Response (POR)

Prof. Dr. Lubna Omar.
Dr. Enas Mohamed Ali.
Dr. Samah Sayed.

Verified by :
dr. Enas

Printing Date : 20/07/2022 10:26:42

Page 2 of 2

Printing By : DR. Israa Elgendy

Pathology Report

Name: **حميدة سلمى السيد**

Age: 43 years.

Clinical data: Right breast cancer.

Specimen type: Right upper quadrerentectomy & axillary clearance.

Date: 17 /5/2020

Lab. Number: 695

Hospital Number: 1575/2020

Gross Description:

Two specimens were received;

A/ Asector of breast tissue; 7x5x4.5 cm, with partial skin covering; 5x3 cm. Serial cuts revealed an ill-defined firm grayish white mass; 2.5x2 cm, located 1.5 cm deep to skin and located (1.5cm, 1.5 cm, 2.5 cm, 2.5 cm) away from (lateral, medial, superior, inferior and deep) margins respectively.

B/ Axillary tissue specimen; 6x4.5 cm entangling 10 nodules up to 2.5x2 cm.

Microscopic Examination:

Histopathological examination of sections prepared from the tumour mass reveals an infiltrating malignant epithelial growth made up of groups and sheets of pleomorphic hyperchromatic malignant ductal epithelial cells. Stromal desmoplasia is moderate. Cellular and nuclear atypia are moderate [grade II]. 5 % intraduct component, micropapillary type could be detected. Positive lymphovascular emboli. There is focal papillary differentiation. Surrounding breast tissue showing fibroadenosis. Four lymph nodes out of 10 dissected lymph nodes showed metastatic deposits [Nodal score is 4/10]. The nipple, areola and surgical margins are free.

Diagnosis:

Right breast, Right upper quadrerentectomy & axillary clearance;

- Invasive duct carcinoma GII, 5% DCIS (micropapillary pattern).
- Free covering skin and resection margins.
- lymph nodes status: 4/10 .
- Stage T2 N2.

Prof. Dr. Gamal Abd el-Aty

Dr. Marya Mahmoud

Prof. Dr. Essam Ayad

Patient Name : الأستاذة / حشوية مسلمة السيد محمد

Patient No. : 27410162600384

Path No. : 2304/21

Sex : Female

Age : 46 Y

Referring Doctor:

Request No. : 138130

Request Date : 03/10/2021

Verification Date : 18/10/2021

Financial Category : برنامج دار السلام - نفقة الدولة

Department : Out Patient

IHC No. :

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:

History of right breast cancer, presented with right axillary enlarged lymph node

Nature of the specimen:

Tru-Cut biopsy

Gross Examination:

greyish white tissue cores, 0.9cm each. Totally submitted.

Microscopic examination:

Histological type: Metastatic poorly differentiated adenocarcinoma compatible with mammary ductal origin, grade III

Diagnosis:

Right axillary lymph node; Tru-Cut biopsy:

Metastatic poorly differentiated adenocarcinoma compatible with mammary ductal origin, grade III

IHC:

ER: No nuclear reaction in tumour cells. Negative, score (0/8)

PR: No nuclear reaction in tumour cells. Negative, score (0/8)

Her-2: No membrane reaction in tumour cells. Negative, score 0

Ci67: 25%

Prof. Dr. Gamal Abd-El Aty

Dr. Abdulrahman Ibrahim



Name : Hamdia semly elsayed

Branch : Portable

Ref.Physician Prof: cap

Date : 29-01-2026 11:27

Medical No. : GRI-12539

Ins.Company :

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

I- CHEST:

- Post right mastectomy status showed bulky right pectoralis major muscle at its medial aspect measuring 38 x 34 mm.
- Large right axillary soft tissue mass lesion measuring 80 x 73 mm showed lobulated margins with bulge into the right supraclavicular region
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

Name : Hamdia semly elsayed

Date : 29-01-2026 11:27

Branch : Portable

Medical No. : GRI-12539

Ref.Physician Prof : cap

Ins.Company :

- ❖ *Post right mastectomy status showed bulky right pectoralis major muscle at its medial aspect ?? infiltrative lesion.*
- ❖ *Large right axillary soft tissue mass lesion ?? nodal*
- ❖ *For PET CT evaluation*

Dr. Asser Ahmed

Dr. Asser Ahmed

Patient Name: <i>Hamdeya Salma Al Sayed Mohamed</i>	Date : <i>23/08/2025</i>
Exam : <i>CT CAP+C (CT Chest Abd. Pelvis)</i>	Age : <i>48 Year(s)</i>
Medical ID: <i>1575/2020</i>	Pat. ID: <i>12539</i>

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST: -

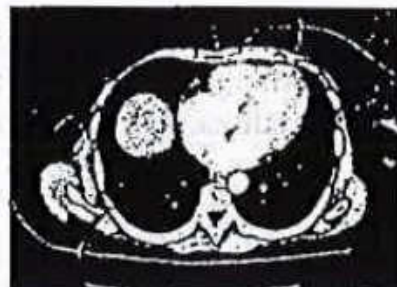
Suboptimal study due to respiratory motion artifact and inadequate breast field.

Clinical History:

- *Known case of right breast cancer, compared to the previous study report dated 23-4-2025, the current study revealed:*

Findings:

- *Clear right MRM operative bed with no sizable soft tissue masses to account for residual/recurrent neoplasia.*
- *Still noted left prominent axillary lymph nodes, likely reactionary.*
- *No sizable nodules, masses or consolidative patches.*
- *Few prominent mediastinal lymph nodes are noted.*
- *Patent tracheo-bronchial tree.*
- *Normal CT appearance of the mediastinal vessels.*
- *Average size of the heart.*
- *No evidence of pleural or pericardial effusion.*
- *Still noted left thyroid lobe nodule with calcification.*



OPINION:

- *Clear right MRM operative bed with no sizable soft tissue masses to account for residual/recurrent neoplasia.*
- *Still noted left prominent axillary lymph nodes.*

Marina Essam, MSc
Dr.Noura Nabil,MD

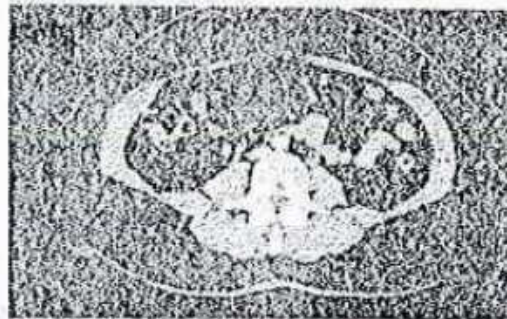
MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS: -

Clinical History:

- *Known case of right breast cancer, compared to the previous study report dated 23-4-2025, the current study revealed:*

Findings:

- The liver is significantly enlarged (15.5 cm) with homogenous CT texture. No sizable focal lesions.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Average size of the spleen (8.5 cm) with no focal lesions seen. Small splenules are still seen.
- Normal appearance of the pancreas and both suprarenal glands.
- Normal size, parenchymal thickness and density of both kidneys with no stones or back-pressure changes.
- Few prominent precaval lymph nodes are noted, the largest seen measuring 10.5x9.5 mm.
- Average filling of the urinary bladder with no sizable stones or masses.



Opinion:

- *Mild hepatomegaly for clinical and lab correlation.*
- *Few prominent precaval lymph nodes.*

Marina Essam, MSc
Dr.Noura Nabil,MD

Signature Noura Nabil
01/09/2025 1:44 AM



Exam Information:

<u>Patient Name:</u> HAMDYAH Selma Elsayed	<u>Date of birth:</u> 16/10/1974
<u>Arabic Name:</u> حمديه سامي السيد	<u>Patient Gender:</u> F
<u>Patient RIS ID:</u> 109057	<u>Exam Date:</u> 29/10/2023
<u>Patient IHS ID:</u> 115011819	
<u>Written by:</u> Dr. Hoda Fathy	<u>Approved by:</u> Dr. Hoda Fathy

Nuclear Medicine Unit

وحدة الطب النووي

^{99m}Tc -MDP BONE SCAN

Brief history: 47 years old female patient diagnosed with managed breast cancer.

Desired Information: - skeletal survey.

Technique:

Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi ^{99m}Tc -MDP in the following manner:

Report:

- ☐ No hot spot that account for osseous deposits could be detected all over the surveyed skeleton.
- ☐ Spondylo-degenerative changes are seen over dorso-lumbar vertebrae.
- ☐ Left maxillary sinusitis.
- ☐ Both kidneys are normally visualized.

Conclusion:

Negative study for the detection of any definite metastatic osseous deposits.

Immunohistochemistry Report

Name: Hamdeya Selmy El Sayed Mohamed

Age: 51 years.

Clinical data: Right breast cancer

Specimen type: TCNB

Date (Received): 30 / 07 / 2026

Date (reported): 20 / 08 / 2026

Lab. Number: 3623/24

Referred by:

Technique:

Sections were prepared from the paraffin block and treated by PT link antigen retrieval then adding monoclonal mouse antibody against PDL-1 (22C3 pharmDX) and HRP detection kit (Dako-EnVision FLEX) using DAB as chromogen and hematoxylin as counter stain.

- PDL-1 was assessed using the combined positive score (CPS)

- CPS = Number of PDL-1 positive cells (tumour cells, lymphocytes & macrophages) total number of viable cells X100.

N.B: Every run containing positive and negative control cell line.

Results:

PDL 1 Combined positive score (CPS) = 7.

Conclusion:

Paraffin block, PDL-1 (22C3) COMBINED POSITIVE SCORE (CPS) = 7, NEGATIVE.

Disclaimer:

- This test reported herein was performed at an external reference lab (Dr. Elia Anis Ishak pathology laboratory), verified by (Prof. Dr. Elia Anis)

